

Use discharge and associated clues

Conjunctivitis is more likely when discharge predominates and vision and the cornea remain normal.

Compare common patterns

Pattern	Supporting clues
Viral	Watery; URTI or contact history; follicles and preauricular node
Simple bacterial	Purulent yellow discharge; lashes and lids may stick together
Allergic	Prominent itch; usually bilateral; atopy and allergen exposure
Severe vernal allergy	Cobblestone papillae, ropery discharge and possible shield ulcer

Look beyond redness

Adenoviral disease often begins in one eye and spreads to the other. Viral cases usually need lubricants and hygiene/contact precautions. Simple bacterial disease is treated with topical broad-spectrum antibiotics. Allergy responds to allergen avoidance and antihistamine or mast-cell stabilising drops. Corneal opacity, photophobia or reduced vision changes the assessment: do not assume that every discharging red eye is uncomplicated conjunctivitis.

Severe or persistent conjunctivitis

Severe or persistent presentations require a more specific diagnosis than simple bacterial conjunctivitis.

Different routes to confirmation

What presentation changes the plan?

Hyperacute, copious milky discharge

Think gonococcus; urgently swab for Gram stain and culture, assess the cornea and admit.

Persistent follicular conjunctivitis

Think Chlamydia, especially with urethral symptoms; obtain a cellular conjunctival scraping for PCR.

Severe allergic corneal disease

Look for a shield ulcer rather than attributing blurred vision to itch alone.

Treat the whole infection

Gonococcal conjunctivitis can ulcerate and perforate the cornea; systemic ceftriaxone and ophthalmic treatment are required. Chlamydial disease requires systemic treatment as well as attention to ocular symptoms. Ask sensitively about genital symptoms and relevant contacts; absence of urethral symptoms does not exclude infection. Severe or atypical disease should be escalated rather than repeatedly treated as simple bacterial conjunctivitis.